

Position statement: Recommendations on the diagnosis and treatment of *Malassezia* folliculitis

Abstract

Malassezia is a lipophilic yeast that is part of the human mycobiome. *Malassezia* folliculitis (MF) occurs when the normally benign colonization of hair follicles by *Malassezia* yeasts becomes symptomatic, manifesting as pruritic papules and pustules. Although MF is commonly encountered in clinical practice, diagnostic and treatment approaches vary among dermatologists and across countries. Therefore, the European Academy of Dermatology and Venereology (EADV) Mycology Task Force *Malassezia* folliculitis working group has developed these recommendations for the diagnosis and management of MF.

The recommendations include diagnostic methods, required positive findings before initiating therapy, and specific treatment algorithms for immunocompetent individuals, immunocompromised patients, and those with compromised liver function. In conclusion, this position statement provides a standardized clinical strategy for the diagnosis and management of *Malassezia* folliculitis.

Introduction

Malassezia (formerly *Pityrosporum*) is a lipophilic yeast commonly found as part of the human mycobiome. Eighteen species have been identified, with *M. furfur*, *M. globosa*, *M. restricta*, *M. sympodialis*, and *M. pachydermatis* most frequently isolated from lesions of *Malassezia* folliculitis (MF). MF arises when

the typically asymptomatic colonization of hair follicles by *Malassezia* becomes pathogenic, leading to pruritic papules and pustules, primarily in seborrheic areas. The condition is often associated with excoriations and post-inflammatory hyperpigmentation.

Common predisposing factors include excessive sweating, hot and humid climates, physical exercise, high sebum production, occlusion from sunscreens or emollients, recent use of antibiotics or corticosteroids, and immunosuppression.

Epidemiological data on the general prevalence of MF are limited. However, hospital-based studies report varying prevalence rates: 1%–2% in China, 3% in Saudi Arabia, 4% in Turkey, and 16% in the Philippines.

Differential Diagnoses

- **Acne vulgaris:** Typically non-pruritic, affects the face and trunk, and is characterized by comedones along with papules and pustules (and sometimes nodules).
- **Acneiform eruptions:** Present with erythematous papules and pustules on the trunk, shoulders, and face; differentiation from MF often requires a punch biopsy.
- **Bacterial folliculitis:** Involves deeper follicular structures and may present with purulent discharge or systemic symptoms such as fever.
- **Eosinophilic folliculitis:** May be associated with elevated peripheral blood eosinophil counts and underlying conditions such as hematologic malignancies or HIV infection.

Given the frequency of MF in clinical dermatology, consistent diagnostic and therapeutic approaches are essential. However, current practices vary widely due to the availability of multiple diagnostic tools and treatment options. To address this variability, the EADV Mycology Task Force working group on MF developed this position statement based on expert consensus to standardize the diagnosis and management of MF.

Materials and Methods

The Expert Committee

The expert committee consisted of dermatologists from the EADV Mycology Task Force working group on *Malassezia* folliculitis, all with specialized expertise in mycology and MF. Conflict of interest disclosures were collected throughout the development of this statement.

Aim and Target Audience

The primary aim was to establish evidence- and consensus-based recommendations for the diagnosis and management of MF. The target audience includes all healthcare professionals involved in the care of patients with MF. The recommendations are presented through diagnostic and therapeutic algorithms to facilitate clinical application.

Literature Review

A systematic literature review was conducted to inform the recommendations. The full methodology and results of this review are provided in the supplementary information.